



1. Identify this tropical disease.
2. Name the causative organism and the vector.
3. Name three drugs used in the treatment. Mention the antibiotic recently advocated in the management of this condition.
4. Name one pulmonary complication of this condition.

PIC-3

1. Filariasis
2. Wuchereria bancrofti , Culex mosquito
3. Ivermectin, DEC, Albendazole. (3)

Doxycycline for treatment against Wolbachia,
an intracellular bacterial symbiont.

4. Tropical pulmonary eosinophilia, chylothorax. (1)



1. Identify the clinical sign?
2. Name two conditions where this sign occurs unilaterally?
3. Name two conditions where this sign occurs bilaterally?
4. Name the commonest cranial nerve involved?

PIC-6

1. Ptosis.
2. Horner's syndrome, unilateral 3rd nerve palsy, Marcus Gunn jaw winking syndrome, congenital simple ptosis, mechanical ptosis.
3. Bilateral 3rd nerve palsy, myasthenia gravis, myotonic dystrophy, snakebite, botulinum toxin, blepharophimosis.
4. Oculomotor nerve.

36 year old female presenting with symmetrical polyarthritis involving small joints, with history of early morning stiffness lasting for 1 hour. O/E patient had the deformity shown below.



1. Identify the deformity and mention the condition in which it occurs.
2. Mention two other hand deformities seen in this disease.
3. Name two cardiovascular manifestations of this disease.
4. Name any two biological agents used in the treatment of the disease.

PIC-7

1. Swan neck deformity, rheumatoid arthritis.
2. Boutennoire deformity, Z deformity, ulnar deviation of wrist, radial deviation of fingers.
3. Pericardial effusion, pericarditis, myocarditis, endocarditis, coronary vasculitis, conduction defects, granulomatous aortitis.
4. Adalimumab, infliximab, etanercept, anakinra, rituximab, abatacept, golimumab, tocilizumab.

Clinical scenario 12

12 years old Mr. Raju, presented with h/o Fever, Breathlessness, Swelling and pain of Right knee joint for 3 days, which resolved spontaneously. Following which patient had swelling of left elbow joint associated with pain. Patient had preceding h/o pain in throat 3 weeks back. O/E Pt. Febrile, PR-98/min, BP-110/70mmHg, CVS-S1, S2+, pansystolic murmur heard in mitral area. Other systems examination normal. TC-14000, P80%, L16%, E2%. ESR 16/30, Hb-12g%, platelets 2.5lac, ASO TITRE initially 100U which risen to 250U. ECG Within normal limits.

1. What is the diagnosis? Name the criteria used for diagnosis?
2. How will you treat this patient?
3. What is the nature of arthritis in this condition?
4. What is Carey Coomb's murmur?

Answers clinical scenario 1

1. Acute Rheumatic Fever(Arthritis with Carditis). Duckett Jones criteria(2 major or 1 major plus 2 minor - along with essential criteria)
2. Aspirin, Benzathine penicillin, Prednisone (if myocarditis is present)
3. Migratory polyarthritides of flitting and fleeting nature involving major joints which subside spontaneously without any residual deformity.
4. Flow MDM across inflamed mitral valve cusps secondary to MR.

PROCEDURE 1:



QUESTIONS:

1. IDENTIFY THE PROCEDURE ??
2. NAME ANY TWO INDICATIONS FOR THIS PROCEDURE ??
3. WHAT IS THE CONTRAINDICATION FOR THIS PROCEDURE ??
4. WHAT ARE THE COMPLICATIONS OF THIS PROCEDURE ??

ANSWERS:

- 1) URINARY CATHETERIZATION**
- 2) ACTUE RETENTION OF URINE , UNCONSCIOUS PATIENTS**
- 3) MEATUS STENOSIS , ACTIVE UTI**
- 4) URETHERAL RUPTURE , IATROGENIC INFECTION , HEAMATURIA**



1. What is the endocrine condition associated with this picture?
2. What is the commonest cause of this condition?
3. Name any two complications associated with this condition.
4. Mention any two tests used for the diagnosis of this condition.

PIC-2

1. Cushings disease /Cushings syndrome
2. Exogenous administration of steroids
3. Hypertension, Diabetes, CAD, central obesity, Osteoporosis, easy bruisability, hirsutism, infertility, cataracts, psychosis, APD, myopathy, secondary infections.
4. Urine free cortisol, overnight dexamethasone suppression test, low dose and high dose dexamethasone suppression test, late evening serum cortisol, diurnal serum cortisol, CRH after dexamethasone test.

26 year old female had the following lesion on her tongue. L/E revealed a white adherent plaque on the tongue and buccal mucosa which bled on attempted removal.



1. Identify the lesion on the tongue.
2. What underlying conditions will you look for?
3. What is the commonest causative organism for this lesion?
4. What is the drug that is recently being advocated when this condition occurs along with HIV infection?

PIC-6

PIC-5

1. Oral candidiasis.
2. HIV, diabetes, inhaled steroids, immunosuppressants, malignancy, tongue piercing, dentures, pregnancy, OCP
3. Candida albicans.
4. Posaconazole.



1. What is the diagnosis?
2. What is the organism causing this disease?
3. Which conditions produce the same lesion extensively in the form of multi dermatomal involvement?
4. Name two complications.

PIC-4

Answers

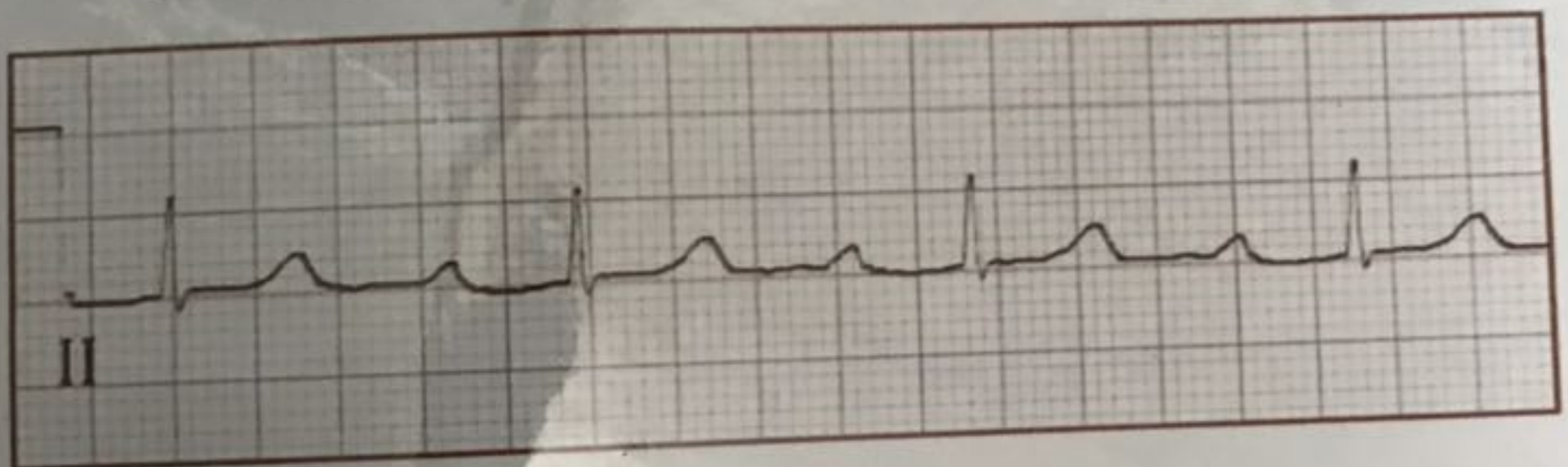
1. Herpes Zoster

2. Varicella Zoster Virus

3. Immune compromised conditions – HIV , Chronic Steroid intake, CKD

4. Herpes Zoster Ophthalmicus , Herpes Zoster Pneumonia
Herpes Zoster Encephalitis

A patient presented to the OPD with complaints of recurrent episodes of fever, sore throat and migratory polyarthrititis. His ECG has the following picture.



1. Comment about the ECG finding.
2. Which criteria is used to diagnose the above said clinical scenario?
3. In which part of the criteria is this ECG finding included?
4. Name the neurological problem that occurs in this disease.

ECG-5

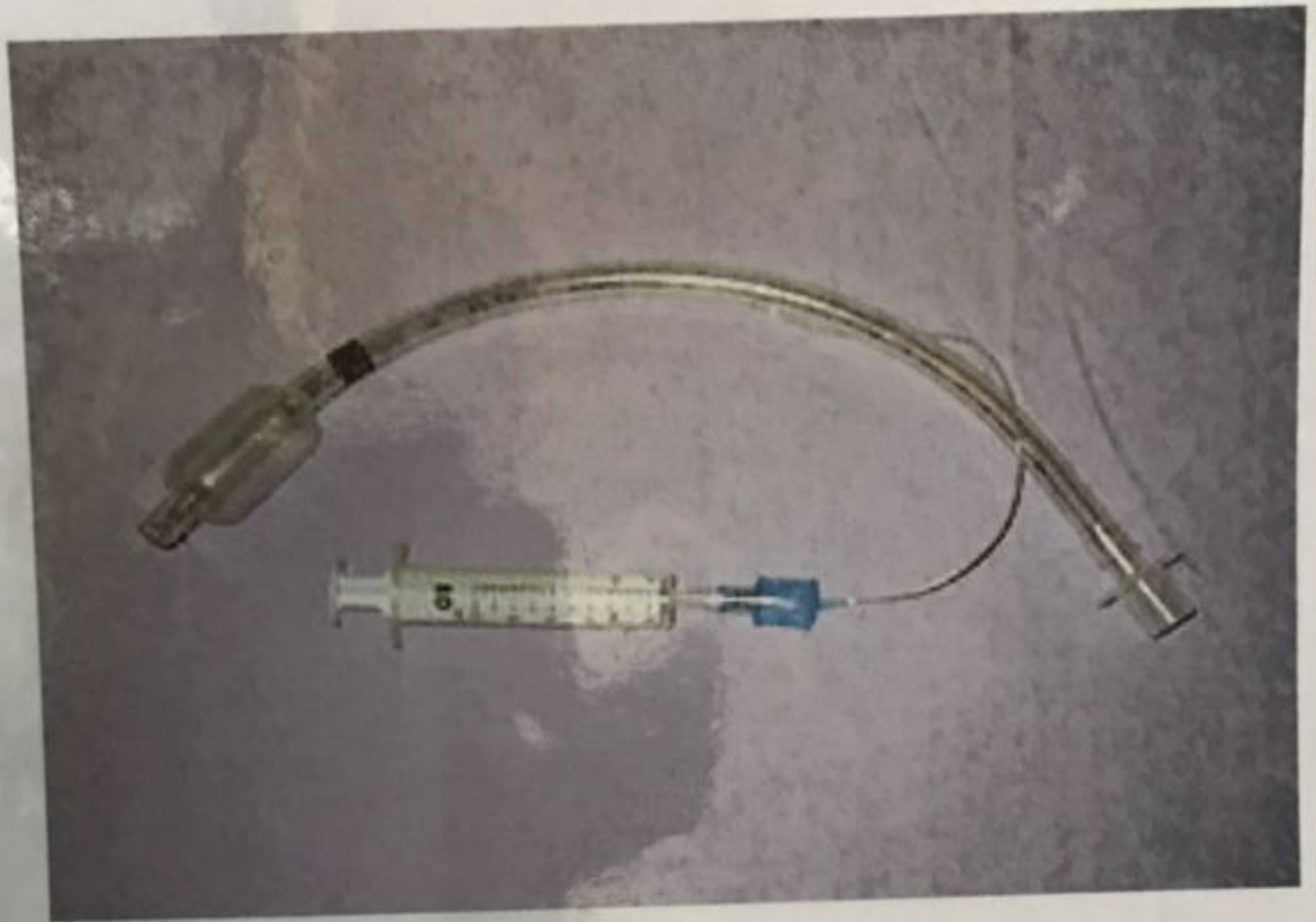
1. Prolonged PR interval.

2. Jones criteria/modified Jones criteria.

3. Minor criteria.

4. Sydenham's chorea.

INSTRUMENT 4



- 1. Identify the tube given.**
- 2. What does the size of the tube indicate?**
- 3. What is the use of the cuff in the tube shown?**
- 4. What is the purpose of the tube shown if it is double lumened?**
- 5. At what level of the Glasgow coma scale is intubation indicated?**

ANSWERS : INSTRUMENT 1

1. CUFFED ENDOTRACHEAL TUBE

2. INDICATIONS:

- a. TO PROTECT AIRWAYS IN UNCONSCIOUS STATE,**
- b. DURING GENERAL ANESTHESIA,**
- c. PTS IN RESPIRATORY DISTRESS,**
- d. FOR PROVIDING POSITIVE PRESSURE VENTILATION.**

3. PREMEDICATIONS:

- a. SEDATIVES: MIDAZOLAM, PROPOFOL**
- b. MUSCLE RELAXANTS : SUCCINYL CHOLINE**

4. BRONCHOSCOPY

cl. ca. 10

Sixty year old Mr. Kumar was admitted with high grade fever, severe headache, projectile vomiting. He had cough with purulent sputum for the past 4 days. On examination, he was found to be febrile and toxic with neck rigidity. He had right lower lobe crepitations and bronchial breathing. Lumbar puncture was done and was reported as follows:

- Appearance : turbid
- Pressure : 320 mm H₂O
- Cells : 100 cells/mm³
- Cytology: predominantly neutrophils.

1. What is the diagnosis?
2. What is the most common organism causing the above disease in healthy immunocompetent individual?
3. If petechial or purpuric lesions with hypotension are found in the above individual, what is the most probable organism?
4. Name any two signs of raised intracranial pressure.

1. Acute bacterial meningitis
2. *Strep. pneumoniae*
3. *N. meningitidis*
4. Reduced or deteriorating level of consciousness, papilledema, dilated or poorly reacting pupils, 6th nerve palsies, decerebrating posture, cushing reflex (bradycardia, hypertension, irregular respiration)

A 40 year old destitute of an institution was found to have loss of appetite and loss of weight over the past three months. He was found to be reasonably active throughout the day but dull and lying down in the evening. One day he became drowsy and was admitted to the nearby hospital.

Examination of the patient revealed neck stiffness, Kernig's sign +ve.

CSF analysis revealed:

Protein	-	104mg%
Glucose	-	20mg%
CSF cytology	-	predominant lymphocytes

Questions:

1. What is the disease condition the clinical examination and CSF analysis denote?
2. What is the characteristic appearance the CSF shows on prolonged standing?
3. What should you rule out before doing lumbar puncture?
4. What is the treatment of this condition?

Answers:

- 1. Tuberculous meningitis**
- 2. Cobweb appearance**
- 3. Papilledema**
- 4. Antituberculous drugs + steroids**

Clinical scenario

Middle aged man known patient of decompensated liver disease who was discharged from the ward two weeks back is brought by his wife with complaints of being sleepy and difficult to arouse. There was history of vomiting for 3 days prior to the altered behaviour.

1. What is the complication of liver failure the patient is suffering from?
2. List two precipitating causes for this condition?
3. Name two signs you could elicit from a patient with this condition?
4. What will the EEG in this condition show?

PROCEDURE 1:

Answers clinical scenario 9

1.	Hepatic encephalopathy
2.	Electrolyte imbalance, infection, constipation, bleeding, protein-rich diet
3.	Asterixis, constructional apraxia, Babinski's sign
4.	Triphasic waves



1. Identify the clinical sign and what is the commonest condition in which it is found?
2. Name two systemic manifestations of this disease.
3. Mention at least two tests to confirm the diagnosis.
4. Name two drugs used in the treatment of the disease?

PIC-1

1. Kayser Fleischer ring – Wilsons disease
2. Acute hepatitis/chronic hepatitis, cirrhosis, extrapyramidal symptoms, dementia, haemolysis, renal tubulopathy, infertility
3. Decreased serum ceruloplasmin, urine copper excretion $>0.6\mu\text{mol}/24\text{hr}$, increased serum copper, copper content of biopsied liver tissue $>250\text{mg/g}$ of liver tissue
Penicillamine, Zinc, Trientine di hydrochloride, tetra thio molybdate.

PROCEDURE 2:



QUESTIONS :

- 1) IDENTIFY THE PROCEDURE ??
- 2) NAME ANY TWO INDICATIONS ??
- 3) WHAT IS SPONTANEOUS BACTERIAL PERITONITIS ??
- 4) WHAT IS TECHNIC TO MINIMIZE THE POST PROCEDURE LEAK ??
should be done after large volume paracentesis